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6.13.2 Acute Aortic Dissection

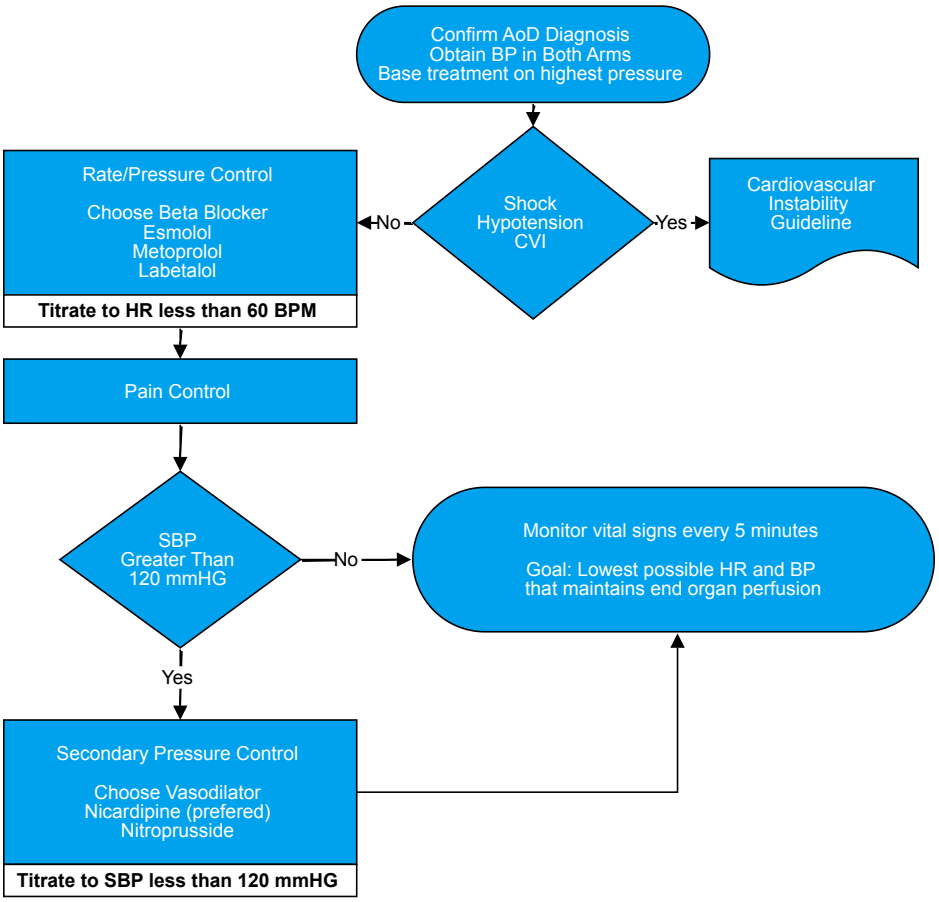
- A well-founded diagnosis should be made by the attending physician prior to implementing therapy in this section.
- Transport management is focused on controlling aortic shear stress to limit false lumen propagation and facilitating timely transport to surgical or endovascular repair.

Assessment

- Obtain a history of current illness with emphasis on chest pain, CV instability, and neurological changes
- Assess pain and ensure adequate analgesia
- Assess and compare central and peripheral pulses for deficits
- Measure and compare blood pressure in both arms
- Titrate treatment based on the higher BP of the 2 arms if there is a discrepancy
- Avoid IV lines in extremities with pulse deficits.
- Review 12 lead ECG, laboratory studies, and imaging reports

Type A dissections affecting the ascending aorta usually require emergent surgical treatment requiring timely preparation and transport.

Initial Management



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Secondary Blood Pressure Control

- Beta blockers alone may not reduce the blood pressure to the target level.
- If the systolic blood pressure remains above 120 mmHg after pain is controlled and the heart rate is consistently <60 BPM, initiate vasodilator therapy.

Goal: Lowest possible heart rate and systolic blood pressure that maintains end organ perfusion.

Vasodilator Therapy

- Target SBP 100 to 120 mm/Hg

Vasodilator therapy without prior beta blockade may cause reflex tachycardia and increased force of ventricular contraction leading to greater aortic wall stress and potentially causing false lumen propagation.

Nicardipine—Preferred

Adult

- 5 mg/hr IV infusion
- Increase by 2.5 mg/hr every 5–15 minutes PRN
- Titrate to BP target
- Max 15 mg/hr

The dose may be reduced to 3 mg/hr after targeted response is achieved

Ped

- No PHI Standing Orders or Indications

Nicardipine Infusion	0.1 mg/mL
Dose mg/hr	**Infusion Rate mL/hr**
3	30
5	50
7.5	75
10	100
12.5	125
15	150
Recommended Mix • 25 mg/250 mL—NS or D5W	

Clevidipine

Adult

- Initiate infusion at 1-2 mg/hr
- Dose may initially be doubled every 90 sec
- As BP approaches goal, reduce the rate of increase by less than double and lengthen time between dose adjustments to every 5–10 min.
- Max dose 32 mg/hr

Ped

- No PHI Standing Orders or Indications

Nitroprusside—Alternate

Adult

- 0.3 to 10 mcg/kg/min
- Start initial infusion at 0.3 mcg/kg/min
- Increase by 0.3 mcg/kg/min Q 5 minutes PRN
- Dose should be kept at 2 mcg/kg/min or less to avoid toxicity
- Titrate to target BP

Pediatric

- Refer to adult dosing

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7.4 Undifferentiated Stroke

Patients with stroke symptoms encountered on scene or in facilities may not have had a head CT or other diagnostic tests to determine if their symptoms are due to ischemic or hemorrhage stroke.

7.4.1 Blood Pressure Management

Patients with stroke symptoms that have not yet had a head CT to determine if their symptoms are due to ischemic or hemorrhage stroke may benefit from blood pressure control.

Acute elevation in blood pressure may be an appropriate response to maintain brain perfusion. Treating hypertension is a balance between the risk of severe BP surges, and the impact on neurologic function when blood pressure is lowered.

If practicable, medical crews should manage blood pressure collaboratively with online medical direction. If medical direction is unavailable, medical crews may treat hypertension under the following conditions and guidelines:

- Positive stroke assessment
- Finger stick blood glucose 60 mg/dL or >
- SBP goal of <185 mmHg

Hypoglycemia can cause neurologic deficits mimicking stroke. It is important to check the blood sugar and rapidly correct low serum glucose.

Nicardipine

Adult

- 5 mg/hr IV infusion
- Increase by 2.5 mg/hr every 5–15 minutes PRN
- Titrate to BP target
- Max 15 mg/hr

The dose may be reduced to 3 mg/hr after targeted response is achieved

Ped

- No PHI Standing Orders or Indications

Clevidipine

Adult

- Initiate infusion at 1-2 mg/hr
- Dose may initially be doubled every 90 sec
- As BP approaches goal, reduce the rate of increase by less than double and lengthen time between dose adjustments to every 5–10 min.
- Max dose 32 mg/hr

Ped

- No PHI Standing Orders or Indications

Labetalol

Adult

- 20 mg IV over 2 minutes
- Repeat 40 mg in 10-20 min PRN
- Repeat 80 mg every 10-20 min up to 3 times PRN
- Alternately 2mg/min IV infusion
- Administer to target BP. HR, or max total dose of 300 mg

Labetalol is a combined alpha and beta blocking agent

Pediatric

- No PHI Standing Orders or Indications

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7.3 Hemorrhagic Stroke

A well-founded diagnosis should be made by the attending physician prior to Medical Crews implementing therapy in this section.

- Perform a neurological exam every 15 minutes and report any significant changes
- Specifically assess for home use or recent administration of anticoagulants

The Medical Crews must manage anticoagulant reversal under direct medical control

Blood Pressure Management

Where practicable, medical crews should manage blood pressure collaboratively with treating providers or medical control. If medical direction is unavailable the medical crews should treat hypertension using the following guidelines:

- Intracranial hemorrhage with SBP > 160 mmHg
- Lower SBP to 130–160 mmHg

Choose one medication for blood pressure reduction:

Nicardipine

Adult

- 5 mg/hr IV infusion
- Increase by 2.5 mg/hr every 5–15 minutes PRN
- Titrate to BP target
- Max 15 mg/hr

Ped

- No PHI Standing Orders or Indications

The dose may be reduced to 3 mg/hr after targeted response is achieved

Clevidipine

Adult

- Initiate infusion at 1-2 mg/hr
- Dose may initially be doubled every 90 sec
- As BP approaches goal, reduce the rate of increase by less than double and lengthen time between dose adjustments to every 5–10 min.
- Max dose 32 mg/hr

Ped

- No PHI Standing Orders or Indications

Labetalol

Adult

- 20 mg IV over 2 minutes
- Repeat 40 mg in 10-20 min PRN
- Repeat 80 mg every 10-20 min up to 3 times PRN
- Alternately 2mg/min IV infusion
- Administer to target BP. HR, or max total dose of 300 mg

Pediatric

- No PHI Standing Orders or Indications

Labetalol is a combined alpha and beta blocking agent

Esmolol

Adult

- Initial load: 500 mcg/kg IV over 1 min
- Titrate 50 mcg/kg/min every 5 min to target HR or BP
- Repeat the loading dose of 500 mcg/kg before every titration PRN until near target HR
- Max infusion dose 300 mcg/kg/min

Pediatric

- No PHI Standing Orders or Indications

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5.1.4 Anxiolysis

- Treat anxiety with titrated benzodiazepines after pain has been controlled with opioids.
- Benzodiazepines used in combination with opioids should be administered with extreme caution and careful patient monitoring.
- Anxious patients can escalate to Dangerous Agitation at any time. Therefore, preparation should be made to respond to Dangerous Agitation.

Indications

- Self reported anxiety
- Evidence of anxious affect

Management

- Treat prior to loading in aircraft if possible

Medications

Midazolam

Adult

- 0.5–5 mg IV over 1-2 min, IM, NAS
- Repeat every 5 min PRN

Pediatric

- 0.05–0.1 mg/kg IBW IV over 1-2 min, IM, NAS
- Maximum single dose 5 mg
- Repeat every 2 min PRN

Lorazepam

Adult

- 0.5 to 2 mg IV over 1 min, IM, NAS
- Repeat every 10-20 min PRN

Pediatric

- 0.05–0.1 mg/kg IV over 1 min, IM, NAS
- Maximum single dose 2 mg
- Repeat every 10-20 min PRN

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Unable to Assess Level of Pain or Sedation

Patient who have received neuromuscular blocking agents or are otherwise unable to be assessed for pain and agitation should be empirically treated using Ketamine mono-therapy or an opioid plus sedative dual agent combination.

Pain and sedation scales such as CPOT, BPS, SAS and RASS, measure behavors that a paralyzed patient cannot display. These scores can not be used to assess and direct analgesia and anxiolysis in patient that have received NMBs.

Multiple or alternating therapeutic approaches may cause adverse events. One pain/sedation approach should be chosen, and then optimized.

Ketamine Mono-therapy

Ketamine

Adult

- 1–2 mg/kg IBW IV over 30–60 seconds
- Repeat bolus dose every 10–20 minutes to achieve the desired sedation goal

To maintain post-Intubation sedation goal after initial IV bolus therapy

- Repeat IV bolus every 20–30 minutes, OR
- 1–4 mg/kg/hr continuous IV infusion

Pediatric

- Refer to adult dosing

Ketamine is the preferred medication for post-intubation analgesia and sedation if the patient is unstable.

Administer one or more initial bolus doses of 1-2 mg/kg IBW IV before starting a ketamine infusion of 1-4 mg/kg/hr. Failing to administer the bolus dose prior to the infusion can lead to under-sedation and dysphoria. The bolus ensures that a dissociative dose is achieved, which is critical for rapid and effective sedation.

Opioid PLUS Sedative Dual Agent Therapy

Administer fentanyl before administering concomitant sedative

Fentanyl

Adult

- 0.5–2 mcg/kg IBW IV over 1 min, IM, NAS
- Repeat every 10 minutes, PRN
- Titrate to sedation goal

Alternatively

- 1-2 mcg/kg/hr via infusion pump

Pediatric

- Refer to adult dosing

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6.13 Vascular Emergencies

6.13.1 Aortic Abdominal Aneurysm (AAA)

A well-founded diagnosis should be made by the attending physician prior to Medical Crews implementing therapy in this section.

- Obtain a detailed history of the present illness, prioritizing symptoms of abdominal pain and signs of cardiovascular instability.
- Administer oxygen as needed, adjusting the flow to maintain a SpO2 between 95% and 99%.
- Continuously monitor cardiac rhythm, oxygen saturation (SpO2), end-tidal CO2 (EtCO2), and non-invasive blood pressure (NIBP).
- Ensure the patient has a venous access line that is appropriate for rapid resuscitation and the administration of blood products if necessary.
- Review the most recent 12-lead ECG, laboratory test results, and imaging studies to inform treatment decisions.

Ruptured AAA with Hypotension and Cardiovascular Instability

Ruptured abdominal aortic aneurysms are acute, life-threatening conditions requiring immediate surgical intervention. These emergencies are highly time-dependent, with outcomes significantly influenced by the speed of diagnosis and surgical response.

Hemorrhage Control

- For ruptured AAA, urgent surgical repair is essential. Transport to a facility capable of emergency vascular surgery, reducing time to definitive care.
- Resuscitation Goals: Begin cautious fluid resuscitation to achieve adequate organ perfusion without exacerbating hemorrhage. Aim for a balance to avoid both over-resuscitation and under-resuscitation.

Titration of Treatment

- Radial Pulse: Use the presence of a palpable radial pulse as an indicator of adequate systolic blood pressure (SBP), adjusting treatment to maintain SBP in the 80-100 mmHg range.
- Normal Mentation: Monitor consciousness and cognitive status to ensure cerebral perfusion. Changes in mentation indicate the need for treatment adjustment.
- Manage Hypothermia, Acidosis, and Coagulopathy: Monitor and address the patient's temperature, acid-base balance, and coagulation to prevent complications that can worsen outcomes.

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7.1 Seizures

Seizures and status epilepticus requires prompt evaluation and treatment. The likely cause of the seizure including brain lesions, medication non-adherence, overdose, toxic exposure, or metabolic disorders should be used to guide therapy.

Seizures due to eclampsia should be managed as per the eclampsia guidelines.
Check a bedside blood glucose and treat hypoglycemia **straightaway.**

- Administer one of the following benzodiazepines every 2–3 minutes until the seizures are controlled.
- IV administration is preferred for treating seizures. Lorazepam IV is preferred, but midazolam IV is an acceptable alternative.

Lorazepam

Adult
- 4 mg IV, IM, NAS

Pediatric
- 0.1 mg/kg IV, IM, NAS
- Max single dose 4 mg

Preferred benzodiazapine if there is IV or IO access.

Repeat every 2–3 minutes until the seizure is controlled.

Midazolam

Adult
- 0.1 mg/kg IV
- Max single dose 5 mg

Alternately
- 0.2 mg/kg NAS, IM
- Max single dose 10 mg

Pediatric
- Refer to adult dosing

Preferred benzodiazapine if IM or NAS administration is indicated.

Repeat every 2–3 minutes until the seizure is controlled.

Neuromuscular blocking agents can mask the motor manifestations of seizures and ongoing status epilepticus; however, they do not treat the underlying condition. These agents should not be used as a treatment for status epilepticus.

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Richmond Agitation Sedation Score (RASS)

Score	Term	Description
4	Combative	Combative or violent, immediate danger to staff
3	Very agitated	Pulls on or removing catheters/tubes or aggressive behavior
2	Agitated	Frequent nonpurposeful movement or ventilator dyssynchrony
1	Restless	Anxious but movements not aggressive or vigorous
0	Alert and calm	
-1	Drowsy	Not fully alert, >10 secs awakening, with eye contact, to voice
-2	Light sedation	Briefly (< 10 secs) awakens with eye contact to voice
-3	Moderate sedation	Any movement (but no eye contact) to voice
-4	Deep sedation	No response to voice, any movement to physical stimulation
-5	Unarousable	No response to voice or physical stimulation

Riker Sedation-Agitation Scale (SAS)

Score	Term	Descriptor
7	Dangerous Agitation	Pulling at ET tube, trying to get off cot, striking at crew, thrashing side-to-side
6	Very Agitated	Requiring restraint and frequent verbal reminding of limits, biting ETT
5	Agitated	Anxious or physically agitated, calms to verbal instructions
4	Calm and Cooperative	Calm, easily arousable, follows commands
3	Sedated	Difficult to arouse but awakens to verbal or physical stimuli, follows commands but drifts off
2	Very Sedated	Arouses to physical stimuli but does not communicate or follow commands, may move spontaneously
1	Unarousable	Minimal or no response to noxious stimuli, does not communicate or follow commands

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Patient Stratification

Stratify the patient using the following categories and apply the applicable part of the guideline:

- Patients whose level of sedation and response to medications **CAN** be assessed and used to guide therapy
- Patients given neuromuscular blockers or whose level of sedation and response to medications **CANNOT** be assessed
- Patients with hemodynamic instability e.g. hypotension, tachycardia, shock index ≥ 1.

Able to Assess Pain and Agitation

- Assess pain and agitation and using an approved self-reporting or observational scale
- Establish a treatment goal
- Use an opioid analgesic or dissociative dose of ketamine
- Use one agent exclusively until it's use and application has been optimized

With the EXCEPTION of Ketamine, use the lower part of the dose range or reduce the normal dose by 1/2 to 1/4 of sedatives if the patient is hemodynamically unstable.

Fentanyl

Adult

- 0.5–2 mcg/kg IBW IV over 1 min, IM, NAS
- Repeat every 10 minutes, PRN
- Titrate to sedation goal

Alternatively

- 1-2 mcg/kg/hr via infusion pump

Pediatric

- Refer to adult dosing

Ketamine

Adult

- 1–2 mg/kg IBW IV over 30–60 seconds
- Repeat bolus dose every 10–20 minutes to achieve the desired sedation goal

To maintain post-Intubation sedation goal after initial IV bolus therapy

- Repeat IV bolus every 20–30 minutes, OR
- 1–4 mg/kg/hr continuous IV infusion

Pediatric

- Refer to adult dosing

Ketamine is the preferred medication for post-intubation analgesia and sedation if the patient is unstable.

Administer one or more initial bolus doses of 1-2 mg/kg IBW IV before starting a ketamine infusion of 1-4 mg/kg/hr. Failing to administer the bolus dose prior to the infusion can lead to under-sedation and dysphoria. The bolus ensures that a dissociative dose is achieved, which is critical for rapid and effective sedation.

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7.5 Brain Herniation Syndrome

Brain herniation may occur due to expanding mass lesions associated with conditions such as intracranial hemorrhage, stroke, abscess, tumors, and hydrocephalus.

- If practicable, medical crews should manage herniation syndrome collaboratively with treating providers or PHI medical directors to ensure decisions are informed by comprehensive medical insights and tailored to the patient's specific needs.
- In cases where medical direction is unavailable, medical crews should follow these guidelines to treat herniation syndrome.

7.5.1 Signs of Brain Herniation Syndrome

- **Neurogenic Breathing Patterns:** Monitor for patterns including Cheyne-Stokes, hyperventilation, and ataxic breathing.
- **Abnormal Pupillary Responses:** Look for abnormalities such as asymmetrical dilation and fixed pupils.
- **Abnormal Motor Responses:** Assess for signs such as decerebrate posturing (extensor posturing), decorticate posturing (flexor posturing), hemiparesis, hemiplegia, and flaccidity.
- **Cushing Reflex:** This reflex is characterized by bradycardia, respiratory depression, and hypertension.

7.5.2 Brain Herniation Syndrome Treatment

Sodium Chloride 3%

Adult

- 250 mL IV over 5-15 min

Ped

- 3–5 mL/kg IBW IV over 5-15 min

Alternate Treatment

Mannitol

Adult

- 0.5-1 gm/kg IV over 30-60 min
- Requires on-line order

Ped

- Refer to adult dosing

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4.9 Hemorrhagic Shock

Control hemorrhage straight away using the techniques described in the trauma guideline.

- Apply direct pressure, pressure dressings, hemostatic dressings, and tourniquets to amendable wounds
- Apply pelvic compression devices for pelvic injuries
- Align and expediently splint long bone fractures

Tranexamic Acid—Uncontrolled Hemorrhage

Adult

- 2 gm IV over 1 min

Pediatric

- 25 mg/kg IV over 1 min
- Max dose 2 gm

May be given over a longer time consistent with local practices
Use adult dosing for 16 and 17-year-old
Use for significant hemorrhage with a SBP < than 90 mm/Hg or heart rate greater than 110 BPM
Administer within 3 hours of injury or bleeding onset

TXA Advisories

Prehospital administration of TXA may not be acceptable in all trauma systems. Medical Crewmembers must be knowledgeable of their regional TXA preferences and operate accordingly.

- Do not mix with blood for transfusion or infusion solutions containing penicillin
- May administer as an infusion—usual mix 2 gm/100 mL (20 mg/mL) in D5W
- Optimal storage temperature 25 °C
- Not indicated in GI hemorrhage

4.9.1 Intravenous Fluid Therapy

IV fluids may be initiated for hemorrhagic shock and hypotension. LR or NS may be used as the lone resuscitation fluid, as blood is being prepared, or used after blood products have been depleted. The therapeutic goals vary depending on the whether the hemorrhage is controlled or uncontrolled.

Small precise crystalloid IV fluids boluses should be administered in a systematic bolus — assess — repeat fashion until the therapeutic goal is archived.

Therapeutic goals

- Uncontrolled hemorrhage—titrate to the presence of a radial pulse and normal mentation
- Controlled hemorrhage— titrate to minimally normalized vital signs

Administration

Administer boluses of Lactated Ringers (LR) or Normal Saline (NS)

- Adult—250-500 mL
- Pediatric—10 to 20 mL/kg
- Newborn Infant—10 mL/kg

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Initial Heart Rate, Blood Pressure and Pain Control

- Treat pain with opiod analgesics
- Administer a beta blocker to reduce the force of ventricular contraction, heart rate and blood pressure.

Goal: Lowest possible heart rate and systolic blood pressure that maintains end organ perfusion.

Beta Blocker Therapy

- Target HR to less than 60 BPM
- Choose 1 of the following:

Esmolol

Adult

- Initial load: 500 mcg/kg IV over 1 min
- Titrate 50 mcg/kg/min every 5 min to target HR or BP
- Repeat the loading dose of 500 mcg/kg before every titration PRN until near target HR
- Max infusion dose 300 mcg/kg/min

Pediatric

- No PHI Standing Orders or Indications

Metoprolol—AMI, Tachyarrhythmias, Vascular Emergencies

Adult

- 5 mg slow IV push over 2–3 min
- Repeat every 5 min PRN
- Max total dose 15 mg

Pediatric

- No PHI Standing Orders or Indications

Labetalol

Adult

- 20 mg IV over 2 minutes
- Repeat 40 mg in 10-20 min PRN
- Repeat 80 mg every 10-20 min up to 3 times PRN
- Alternately 2mg/min IV infusion
- Administer to target BP. HR, or max total dose of 300 mg

Labetalol is a combined alpha and beta blocking agent

Pediatric

- No PHI Standing Orders or Indications

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6.3 At Risk for Dangerous Agitation

Indications

- Behavior that interferes with crew duties and therapies
- Physical evidence of anxiety (e.g. restlessness, panic)
- Intoxication (alcohol or drugs)
- Withdrawal symptoms from substances
- History of anxiety or delirium
- Other concerning behaviors or signs

Management

- Treat prior to loading in the aircraft if possible.
- Consider notifying the pilot of risk

Further Preparation

Anxious patients and patients at risk for Dangerous Agitation can escalate to Dangerous Agitation at any time. Therefore preparations should be made to respond to Dangerous Agitation as Follows:

Preparations required at Head Of Bed:

- Appropriately sized I-gel
- Appropriately sized BVM kit
- Appropriate medications
- Ensure availability of all airway management equipment and appropriate personnel

Airway management must be performed if patient's ventilatory status becomes compromised and is mandatory with administration of neuromuscular blockade.

Droperidol—At Risk for Dangerous Agitation

Preferred initial therapy for adults.

Adult

- 2.5 mg IV, IM, or IO
- Repeat in 3 minutes if needed
- Maximum 10 mg

Maximum effects may not occur for 20–30 minutes

May be used as monotherapy or in combination with a benzodiazepine

Consider a 50% dose reduction in elderly patients or those with significant comorbidities to reduce the risk of respiratory depression and prolonged sedation.

Haloperidol may be used as an alternative agent to Droperidol.

Ketamine may not work as well in the agitated alcohol withdrawal patient or the acutely stimulant intoxicated patient. In these patients, every 5 minute dosing of benzodiazepines with coadministration of an antipsychotic is preferred.

There are no pediatric indications or standing orders for At Risk Dangerous Agitation. Administration requires a physician order.

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